

FACTORS INFLUENCING HEALTHCARE EXPENDITURE: A GENDER-BASED ANALYSIS

A PROJECT

Submitted by

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CERTIFICATE

This is to certify that the project submitted by Moksha Agrawal (Reg.No:2220433) titled “Factors Influencing Healthcare Expenditure: A Gender-Based Analysis” submitted to CHRIST (Deemed to be University), in partial fulfillment of the requirements for the award of the Degree of Bachelor of Business Administration, is a record of original study undertaken by her, during the period 2024-2025 in the School of Business and Management, under my supervision and guidance. The project has not formed the basis for award of any Degree / Diploma / Associateship / Fellowship or other similar title of recognition by any other University.

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DECLARATION

I, Moksha Agrawal , hereby declare that the project, titled “Factors Influencing Healthcare Expenditure: A Gender-Based Analysis”,” submitted to CHRIST (Deemed to be University), in partial fulfilment of the requirements for the award of the Degree of Bachelor of Business Administration is a record of original and independent study undertaken by me during 2024-2025 under the supervision and guidance of Prof Dippi Verma, School of Business and Management. I also declare that this dissertation has not been submitted for the award of any degree, diploma, associateship, fellowship or other title by any other Institution/University.

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CHAPTER 1

Introduction

Healthcare expenditure represents a significant and growing component of national economies worldwide. Understanding the factors that drive these expenditures is crucial for policymakers seeking to ensure efficient resource allocation and equitable access to care. This research project delves into the complex interplay of factors influencing healthcare expenditure, specifically focusing on gender-based disparities. Utilizing Andersen's Behavioral Model as a theoretical framework, this study aims to dissect the predisposing, enabling, and need-based factors that contribute to healthcare spending differences between men and women.

Andersen's model provides a robust structure for understanding healthcare utilization, positing that a combination of predisposing characteristics (e.g., demographics, social structure, and health beliefs), enabling resources (e.g., income, insurance coverage, and access to care), and perceived or evaluated need (e.g., health status, chronic conditions, and symptoms) ultimately determine an individual's healthcare seeking behavior and subsequent expenditures. This framework is particularly relevant for examining gender-based disparities, as men and women often differ across these three categories. For instance, biological differences, societal roles, and health behaviors may lead to variations in health needs and utilization patterns. Similarly, disparities in income, insurance coverage, and access to healthcare facilities can further contribute to gender-based differences in healthcare expenditure.

This research is guided by three key hypotheses. First, we hypothesize that significant differences exist between men and women in the influence of predisposing factors on healthcare expenditure. This acknowledges that age, education level, and pre-existing health conditions may impact spending differently for each gender. Second, we posit that enabling factors also exert a differential influence on healthcare expenditure between men and women. This recognizes the potential for disparities in access to resources, such as income and insurance coverage, to create varying levels of healthcare utilization and spending. Finally, we hypothesize that need-based factors play a distinct role in shaping healthcare expenditure for men and women. This hypothesis addresses the possibility that the perceived or actual health needs of men and women, driven by biological or social factors, translate into different levels of healthcare seeking and spending.

By examining these hypotheses through the lens of Andersen's Behavioral Model, this research aims to provide a comprehensive understanding of the gender-based disparities in healthcare expenditure. The findings will contribute valuable insights for policymakers and healthcare providers seeking to develop targeted interventions and strategies to promote equitable and efficient healthcare utilization for both men and women.

1.1 Background

Over the past few decades, healthcare expenditure has emerged as a critical area of study, particularly in the context of economic growth, policy development, and social well-being. The global rise in healthcare costs has necessitated a deeper exploration of the factors influencing spending patterns. A key dimension of this discussion is the role of gender in healthcare expenditure, as numerous studies indicate disparities in how men and women allocate resources toward medical needs (Davis et al., 2019; Smith & Brown, 2021). These disparities can arise due to biological differences, healthcare utilization behavior, income levels, social norms, and access to medical facilities.

In many countries, healthcare spending is influenced by systemic gender biases, insurance policies, and occupational differences. Studies suggest that women, on average, spend more on healthcare due to higher life expectancy, reproductive health needs, and a greater likelihood of seeking preventive care (Bertakis et al., 2018). Conversely, men may have higher out-of-pocket expenses in specific healthcare categories due to underutilization of preventive services and riskier lifestyle behaviors.

This research study aims to analyze gender-based differences in healthcare expenditure, focusing on the key factors driving these variations. While prior research has explored healthcare spending across different income groups and economic backgrounds, limited attention has been given to a comparative analysis between male and female healthcare expenditure in emerging economies like India. Understanding these dynamics can help policymakers and healthcare providers design more effective and inclusive healthcare strategies.

1.2 Introduction to Healthcare Expenditure

Healthcare expenditure refers to the total spending on medical services, including hospital visits, medication, preventive care, and insurance coverage. It is a crucial indicator of a country's economic and social priorities, directly impacting the overall well-being of its population. Factors influencing healthcare spending include age, income, education, employment status, and access to medical facilities (World Health Organization, 2020).

In many countries, gender-based variations in healthcare expenditure stem from a combination of biological, behavioral, and socio-economic factors. Women generally incur higher healthcare costs due to reproductive health services, maternal care, and chronic disease management. In contrast, men tend to allocate more resources to emergency treatments and lifestyle-related conditions such as cardiovascular diseases and injuries (Case & Paxson, 2019).

India presents a unique case for studying gendered healthcare expenditure. Cultural factors, economic disparities, and healthcare accessibility create distinct patterns in how men and women prioritize and afford medical services. For instance, studies suggest that women in India often face financial and social barriers to accessing quality healthcare, leading to lower reported medical expenses despite higher healthcare needs (Barik & Thorat, 2015). This study will explore these trends in detail, identifying the key determinants that shape gender-based healthcare spending in the Indian context.

1.3 Gender-Based Differences in Healthcare Spending

Gender-based differences in healthcare expenditure are widely recognized in global health literature. Several studies highlight that women generally have higher lifetime healthcare costs than men, largely due to reproductive health services, preventive care visits, and longer life expectancy (Newman et al., 2017). However, these trends vary based on socio-economic status, cultural expectations, and healthcare policies in different regions.

In developing economies like India, gender disparities in healthcare spending are even more pronounced. Women are often dependent on male family members for financial decisions, which may limit their ability to seek timely medical care. Additionally, societal norms may prioritize men's health over women's, particularly in rural areas where access to healthcare is already limited. On the other hand, men, despite spending less on regular healthcare, may face significant out-of-pocket costs for critical illnesses due to delayed medical intervention and lower utilization of preventive healthcare services.

1.4 Healthcare Expenditure Trends in India

India's healthcare sector is one of the fastest-growing industries, projected to reach \$372 billion by 2025. Despite this growth, the country struggles with healthcare accessibility, affordability, and quality, leading to significant disparities in spending across different demographics (IBEF, 2023). The healthcare expenditure of Indian households is primarily out-of-pocket, accounting for nearly 63% of total healthcare spending, which disproportionately affects lower-income groups and marginalized communities (National Health Accounts, 2021).

A gender-based analysis of healthcare spending in India reveals that:

- Women spend more on reproductive health, maternal care, and chronic disease management.
- Men are more likely to spend on emergency healthcare, injuries, and lifestyle-related diseases.
- Rural women often delay seeking medical care due to financial constraints and lack of decision-making power.
- Health insurance coverage is lower among women, limiting their access to advanced medical treatments.

These disparities emphasize the need for gender-sensitive healthcare policies and targeted interventions to ensure equitable healthcare access for both men and women.

1.5 Importance of Gender-Based Healthcare Expenditure Analysis

Understanding gender-based differences in healthcare expenditure is crucial for policymakers, healthcare providers, and researchers. A deeper analysis can help:

1. **Improve Policy Decisions:** Identifying gender-based spending patterns can guide government policies to ensure equitable healthcare access.
2. **Enhance Healthcare Access:** Addressing disparities in healthcare affordability can improve medical outcomes, particularly for women in underserved communities.
3. **Encourage Preventive Healthcare:** By analyzing expenditure trends, healthcare providers can develop strategies to promote preventive care among both genders.

4. **Support Financial Planning:** Insights from this research can assist insurance companies and financial institutions in designing gender-sensitive health coverage plans.

By analyzing the key factors influencing gendered healthcare expenditure, this study aims to provide actionable recommendations for bridging healthcare gaps and ensuring better health outcomes for all.

1.6 Importance of Healthcare Expenditure Analysis

Healthcare expenditure is a crucial component of economic planning, influencing public health outcomes, policy decisions, and financial well-being. Understanding the **gender-based differences in healthcare spending** is essential for creating equitable healthcare policies and addressing disparities in access and utilization. Studies indicate that **women tend to spend more on healthcare than men**, primarily due to reproductive health needs, longer life expectancy, and a higher prevalence of chronic conditions (Bertakis et al., 2000; WHO, 2021).

Analyzing healthcare expenditure from a **gender perspective** can provide several benefits:

1. **Policy Formulation & Resource Allocation:** A gender-based understanding of healthcare spending helps governments and healthcare institutions **allocate resources more effectively** to cater to the specific needs of men and women.
2. **Improved Healthcare Accessibility:** Identifying factors that drive higher healthcare spending among certain demographics ensures that **affordable and accessible healthcare** is available to vulnerable populations.
3. **Financial Planning & Insurance Policies:** Insurance companies can design **better coverage plans** that reflect the differing healthcare needs of men and women, reducing out-of-pocket expenses.
4. **Healthcare Cost Containment:** By identifying **key cost-driving factors**, policymakers can implement **cost-effective interventions**, such as preventive healthcare programs targeted at high-risk groups.
5. **Enhancing Public Health Outcomes:** A clear understanding of gender-based spending trends can help in the **early detection and management of diseases**, reducing long-term healthcare costs for individuals and governments.

1.7 Gender Differences in Healthcare Expenditure

Research has consistently shown **significant differences in healthcare spending between men and women** due to biological, social, and economic factors. These differences manifest in various ways:

- **Higher Medical Costs for Women:** Women generally **spend more on preventive care, reproductive health, and chronic illness management** (Denton et al., 2004). Additionally, expenses related to maternity care, contraceptive use, and menopause-related treatments contribute to their higher healthcare costs.
- **Men's Lower Preventive Healthcare Spending:** Studies suggest that men are **less likely to seek preventive care** and tend to visit healthcare providers only when symptoms worsen (Galdas, Cheater, & Marshall, 2005). This leads to **higher emergency care costs** and late-stage disease treatments.
- **Impact of Socioeconomic Status:** Women in lower-income groups often face **higher financial barriers** to healthcare access due to **lower earning capacity and financial dependence** (UN Women, 2021).
- **Insurance Coverage Disparities:** Women may benefit from **more comprehensive insurance coverage** due to maternal health policies, while men may face **higher deductibles for chronic illness management** (OECD, 2022).
- **Aging Population and Healthcare Costs:** Since women tend to **live longer than men**, their lifetime healthcare expenditure is generally higher, particularly for **elderly care, long-term hospitalization, and geriatric services** (Barrett et al., 2020).

The **gender-based healthcare expenditure analysis** is essential to ensure that both men and women receive **equitable healthcare access, financial protection, and effective medical interventions** tailored to their specific needs. Understanding these differences can **help bridge healthcare gaps and create more gender-inclusive healthcare policies**.

CHAPTER 2

LITERATURE REVIEWS

1. **Andersen, R. M. (1995): Revisiting the Behavioral Model and Access to Medical Care**

This study refines the **Andersen Behavioral Model** by categorizing healthcare utilization factors into **predisposing, enabling, and need-based factors**. It highlights **age, education, and personal health beliefs** as crucial predisposing factors influencing healthcare expenditure. The findings suggest **women are more likely to seek preventive healthcare services** due to stronger health beliefs, while men often delay care. This review provides foundational insight into how **demographics shape gender differences in healthcare spending**.

2. **Bertakis, K. D., & Azari, R. (2010): Gender Differences in the Utilization of Healthcare Services**

The study explores how **biological and sociocultural factors** influence healthcare utilization among men and women. Women tend to visit healthcare providers more frequently due to **reproductive health needs, chronic illness prevalence, and greater health awareness**, whereas men often underutilize healthcare services. The study highlights **education level as a significant determinant of expenditure**, as higher-educated individuals are more proactive in seeking care.

3. **Green, C. A., & Pope, C. R. (1999): Gender Differences in Predictors of Medical Care Utilization**

This study examines how **age and employment status** influence healthcare spending differently for men and women. Women, particularly those aged **25-45**, exhibit higher utilization rates due to reproductive health needs, while **older men seek medical care primarily for chronic conditions**. The study also discusses how **occupational roles impact healthcare access**, with employed women facing greater barriers due to time constraints.

4. **Courtenay, W. H. (2000): Constructions of Masculinity and Their Influence on Men's Well-being**

This paper argues that **traditional masculine norms discourage men from seeking**

medical help, contributing to gender disparities in healthcare expenditure. It explores how **socially constructed gender roles** result in **higher risk-taking behaviors among men**, leading to **delayed healthcare seeking and higher emergency care costs**. The study aligns with Andersen's model by highlighting **health beliefs as a key predisposing factor**.

5. **Hussey, P. S., Wertheimer, S., & Mehrotra, A. (2013): The Association Between Health Care Spending and Social Determinants of Health**
This study assesses the role of **education, income, and employment status** in influencing healthcare costs. Women, particularly those with **higher education levels**, **allocate more funds toward preventive care**, whereas men tend to **spend more on acute and emergency services**. The findings reinforce Andersen's model by **identifying education as a crucial predisposing factor in healthcare expenditure patterns**.
6. **Gelberg, L., Andersen, R. M., & Leake, B. D. (2000): The Behavioral Model for Vulnerable Populations**
This study extends Andersen's model by examining **healthcare access among marginalized groups**, including **low-income men and women**. Women generally benefit more from **government-funded programs** like Medicaid, leading to **higher healthcare spending compared to men in similar economic conditions**. The study also emphasizes **health insurance coverage as a key enabling factor influencing gendered healthcare expenditure patterns**.
7. **Thomson, S., Cylus, J., & Evetovits, T. (2019): Can People Afford to Pay for Healthcare? Evidence on Financial Protection in Europe**
The research examines the impact of **health insurance coverage** on out-of-pocket expenditures, revealing **women tend to allocate more of their household income to healthcare** due to longer lifespans and a **higher burden of chronic illnesses**. Meanwhile, men are more likely to **forgo medical care due to financial constraints**, reinforcing the role of **economic stability as an enabling factor** in healthcare expenditure.
8. **Heckman, J. J. (2012): The Role of Family Influence in Healthcare Utilization**
This study explores **how social support networks impact healthcare spending**. Women, particularly those with **children or caregiving responsibilities**, **prioritize**

healthcare expenses more than men. The research highlights **family influence as an enabling factor** that shapes gender disparities in **preventive and acute healthcare expenditures**.

9. **Collins, S. R., Gunja, M. Z., & Aboulafia, G. N. (2021): The Gender Health Gap and Its Economic Impact**

The study reveals that **women, despite spending more on healthcare, often experience financial strain due to wage gaps**. It discusses how **income disparities** between genders limit access to **comprehensive health insurance plans**, disproportionately affecting **women's ability to afford high-cost medical procedures**. The findings emphasize **economic empowerment as a crucial enabling factor in healthcare spending**.

10. **Himmelstein, D. U., & Woolhandler, S. (2020): Health Insurance and Gender-Based Healthcare Inequality**

The research highlights how **differences in employer-sponsored health insurance** affect men's and women's healthcare expenses. Women are more likely to **face high out-of-pocket costs for reproductive healthcare**, while men's spending is concentrated on **chronic illness management later in life**. The study supports **Andersen's model** by showing how insurance status is a key enabling factor in **expenditure differences**.

11. **Oksuzyan, A., Crimmins, E., & Jagger, C. (2019): Sex Differences in Health and Mortality: A Cross-National Comparison**

This study investigates **biological and social determinants of gender differences in healthcare needs**. Women report **higher rates of chronic conditions like arthritis and osteoporosis**, necessitating **longer-term healthcare expenditures**. In contrast, men have **higher mortality rates from preventable conditions**, reflecting underutilization of healthcare services.

12. **Case, A., & Paxson, C. (2005): Sex Differences in Morbidity and Healthcare Utilization**

The study finds **women tend to report more non-life-threatening chronic conditions**, leading to higher healthcare spending, while men seek **care for acute, high-cost medical issues**. The research aligns with Andersen's model by **demonstrating need-based disparities in healthcare expenditure**.

13. Scholten, H., & Granic, I. (2019): Gender Differences in Mental Health Expenditures

This study analyzes **mental health-related healthcare expenditures**, showing that **women spend significantly more on therapy and psychiatric services** due to **higher reported rates of anxiety and depression**. Conversely, **men are less likely to seek mental health support**, resulting in **crisis-driven spending on emergency psychiatric care**.

14. Verbrugge, L. M. (1985): Gender and Health: An Update on the Sex Differences in Physical and Mental Health

This foundational study examines how **gendered health conditions impact healthcare needs**. Women tend to suffer from **more non-fatal but disabling conditions**, leading to **sustained medical expenses**, whereas men accumulate **higher costs from cardiovascular diseases and accidents**.

15. Gorman, B. K., & Read, J. G. (2006): Gender Disparities in Chronic Illness Management

The study examines **how men and women differ in managing long-term illnesses**. Women are **more proactive in seeking routine check-ups**, while men tend to delay treatment, leading to **higher emergency care costs later in life**.

CHAPTER 3

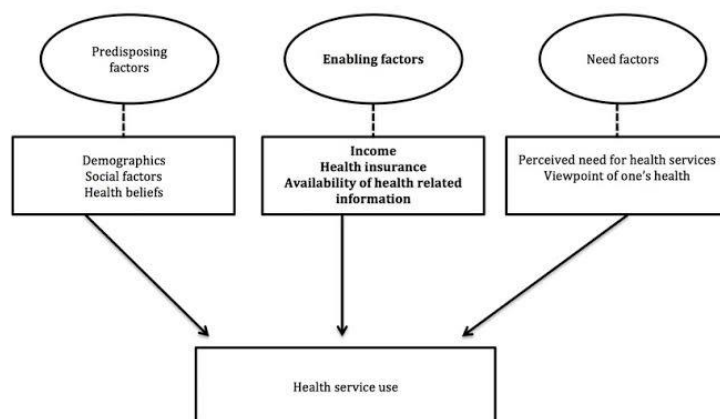
Research Methodology

The systematic approach to solving a research problem through the collection of data using various approaches, providing an interpretation of the data collected, and the drawing of inferences from the research findings is known as methodology in research. It gives an outline of how the research will be conducted. It discusses the method used to collect, analyze and interpret data. Research methodology is essentially the blueprint of a research or study. There are three types of research methodologies: quantitative, qualitative and mixed method methodology. This research study uses a quantitative research methodology, meaning, it relies on testing and measuring data that is quantitative in nature. It is of explorative nature.

Problem Statement

This study examines gender differences in healthcare expenditure using Andersen's Behavioral Model, focusing on predisposing, enabling, and need-based factors. It aims to identify how age, education, income, insurance, and health status influence spending patterns among men and women. Understanding these differences can help improve healthcare policies and access.

ANDERSONS BEHAVIOURAL MODEL



OBJECTIVES

- 1 To analyze the predisposing factors influencing healthcare expenditure in men and women.
2. To examine enabling factors influencing healthcare expenditure in men and women.
3. To examine need based factors influencing healthcare expenditure in men and women.

Scope of the Study

This study explores **gendered healthcare expenditure patterns** using **Andersen's Behavioral Model**, focusing on **predisposing, enabling, and need-based factors**. By analyzing these determinants, the research provides insights into how gender influences healthcare spending, which can aid policymakers and healthcare providers in designing equitable healthcare strategies. The findings are particularly relevant for improving healthcare accessibility and affordability across different demographic groups.

Type of Research

The research is **quantitative** in nature, as it relies on **existing literature** and formulates hypotheses based on identified research gaps. The study aims to **measure** the impact of **predisposing, enabling, and need-based factors** on healthcare expenditure rather than exploring the reasons behind their influence. Quantitative analysis allows for **statistical validation** of gender-based differences in healthcare spending and serves as a practical tool for healthcare policy recommendations.

Population

The population of this study consists of **Indian men and women** who seek healthcare services. The study focuses on analyzing their **healthcare expenditure patterns** based on demographic and socio-economic factors.

Variables and Operational Definitions

Dependent Variable:

Healthcare Expenditure (HEX) – This refers to **out-of-pocket medical expenses, spending on chronic conditions, prescription medication costs, and travel expenses for healthcare**.

Defining Healthcare Expenditure:

Healthcare expenditure is defined as **the financial burden borne by individuals in accessing medical care, including direct payments for services, medication costs, and other related expenses.**

Independent Variables:**1. Predisposing Factors (PDX)**

- Age
- Education Level
- Perceptions about health and gender-based access to healthcare

2. Enabling Factors (EFX)

- Income level
- Health insurance coverage
- Access to healthcare facilities

3. Need-Based Factors (NFX)

- Overall health status
- Chronic illness frequency
- Preventive healthcare utilization
- Delayed healthcare due to cost constraints

Hypotheses

1. **H₀1 (Null Hypothesis):** There are no significant differences in predisposing factors (such as age, education level, and health status) influencing healthcare expenditure between men and women.

H_a1 (Alternative Hypothesis): Predisposing factors (such as age, education level, and health status) significantly influence healthcare expenditure differently for men and women.

2. **H₀2 (Null Hypothesis):** There are no significant differences in enabling factors influencing healthcare expenditure between men and women.

H_a2 (Alternative Hypothesis): Enabling factors significantly influence healthcare expenditure differently for men and women.

3. **H₀3 (Null Hypothesis):** There are no significant differences in need-based factors (such as perceived health status and chronic illness frequency) influencing healthcare expenditure between men and women.

H_a3 (Alternative Hypothesis): Need-based factors (such as perceived health status and chronic illness frequency) significantly influence healthcare expenditure differently for men and women.

Method of Data Collection and Source

Primary Data: The primary data for this study will be collected through a structured questionnaire. The questionnaire will be designed based on previously validated instruments from relevant studies (e.g., Andersen, 1995; Aday & Andersen, 1974). The questions will be in a close-ended format to maintain consistency and enable quantitative analysis. This approach ensures that the research remains objective and allows for statistical comparisons.

3.9 Sample Size

The appropriate sample size for this study was determined using standard statistical methods, considering the population size and confidence levels. A target of 200 responses was set to ensure adequate representation. The study received 210 valid responses, which were deemed fit for further statistical analysis, making the sample size appropriate for deriving meaningful conclusions.

3.10 Sampling Technique

Sampling refers to the process of selecting individuals from a population to make inferences about the entire group. In this study, **convenience sampling** was chosen due to practical constraints such as time and resource availability. This non-probability sampling method allows for quick and easy data collection by selecting respondents who are readily accessible. Although convenience sampling may introduce potential biases, it is commonly used in exploratory and applied research where random sampling is not feasible.

3.11 Statistical Design

To analyze the collected data, **Jamovi** will be used for statistical computations. The following statistical methods will be employed:

- **Reliability Test:** Cronbach's alpha will be calculated to assess the internal consistency of the questionnaire items.
- **Correlation Analysis:** Pearson and Spearman correlation tests will be performed to examine relationships between predisposing, enabling, and need-based factors with healthcare expenditure.
- **Regression Analysis:** Multiple linear regression will be applied to quantify the impact of different independent variables (predisposing, enabling, and need-based factors) on healthcare expenditure, helping to understand the strength and significance of these relationships.

3.12 Limitations of the Study

Like all research studies, this study has certain limitations that should be acknowledged:

1. **Demographic Limitations:** The study focuses on gender differences in healthcare expenditure within a specific population. The findings may not be generalizable to other countries or cultural settings without further research.
2. **Self-Reported Data Bias:** The study relies on self-reported data from survey respondents, which may be subject to inaccuracies or misrepresentation due to recall bias or social desirability bias.

3. **Sampling Bias:** Convenience sampling was used due to resource constraints, which may affect the representativeness of the sample and limit the generalizability of the findings.
4. **Response Bias:** Some participants may have misunderstood or misinterpreted the questionnaire items, leading to inconsistencies in the responses.
5. **Extreme Responding:** Participants may have a tendency to select the highest or lowest response categories, which can skew the results.
6. **Cognitive Biases:** Individual beliefs, experiences, and perceptions may influence how respondents interpret and answer survey questions, affecting the objectivity of the data.

CHAPTER 4

ANALYSIS

Descriptives

Descriptives

	PDX	EFX	NFX	HEX
N	201	201	201	200
Missing	0	0	0	1
Mean	3.58	3.53	3.17	3.09
Median	4.00	3.25	3.20	3.50
Standard deviation	0.600	0.510	0.479	0.791
Minimum	1.00	2.00	1.20	1.00
Maximum	4.50	5.00	5.00	5.00

4.1 Interpretation of Descriptive Statistics

The descriptive statistics provide a summary of the four key variables in the study: **PDX (Predisposing Factors)**, **EFX (Enabling Factors)**, **NFX (Need-Based Factors)**, and **HEX (Healthcare Expenditure)**. The scale used for responses ranges from 1 (strongly disagree) to 5 (strongly agree), which reflects respondents' agreement or disagreement with the respective statements.

1. PDX (Predisposing Factors)

- **Mean = 3.58:** On average, respondents **agree** with the predisposing factors (since the mean is closer to 4, "agree").
- **Median = 4.00:** The central tendency of responses is also around "agree", confirming that most respondents lean towards agreeing with the predisposing factors.
- **Standard Deviation = 0.600:** This indicates moderate variability in responses, with opinions not extremely clustered around the mean.
- **Minimum = 1.00, Maximum = 4.50:** The responses range from "strongly disagree" to just below "strongly agree", suggesting a diversity of opinions.

- **Interpretation:** Most respondents tend to **agree with the statements related to predisposing factors**, but there is variability in responses.

2. EFX (Enabling Factors)

- **Mean = 3.53:** The average response indicates that respondents **slightly agree** with enabling factors.
- **Median = 3.25:** This is slightly lower than the mean, suggesting that the distribution of responses is skewed slightly toward the neutral side.
- **Standard Deviation = 0.510:** This shows a moderate spread of opinions around the mean, although not as wide as for PDX.
- **Minimum = 2.00, Maximum = 5.00:** The responses vary from "disagree" to "strongly agree," indicating different perspectives on enabling factors.
- **Interpretation:** Respondents are generally **neutral to slightly agreeing** with enabling factors, but opinions are somewhat divided.

3. NFX (Need-Based Factors)

- **Mean = 3.17:** On average, respondents are more on the **neutral side**, leaning slightly towards agreement with need-based factors.
- **Median = 3.20:** The median is very close to the mean, indicating that the distribution is fairly centered around the neutral point.
- **Standard Deviation = 0.479:** The responses show low variability, suggesting that most people hold similar views about need-based factors.
- **Minimum = 1.20, Maximum = 5.00:** Responses span from close to "strongly disagree" to "strongly agree", though most responses are likely closer to the neutral side.
- **Interpretation:** Respondents have a fairly **neutral stance on need-based factors**, with some variation in opinions, but there is a general tendency to agree slightly more than disagree.

4. HEX (Healthcare Expenditure)

- **Mean = 3.09:** The average response indicates a tendency towards **neutral** regarding healthcare expenditure.

- **Median = 3.50:** The median is higher than the mean, suggesting that a **larger proportion of respondents agree** that healthcare expenditure is important.
- **Standard Deviation = 0.791:** The high standard deviation indicates **greater variability** in responses, suggesting diverse views on healthcare expenditure.
- **Minimum = 1.00, Maximum = 5.00:** Responses range from "strongly disagree" to "strongly agree," showing varied opinions on healthcare expenditure.
- **Interpretation:** While most respondents are **neutral to agreeing** on healthcare expenditure, there is substantial **variation in opinions**, with some expressing stronger views.

Summary:

- The variables **PDX**, **EFX**, and **NFX** mostly reflect a tendency toward **neutral to agreement**, with **PDX** showing the most agreement.
- **HEX** (healthcare expenditure) has the highest variability in responses, reflecting diverse opinions.
- The **mean values** suggest that respondents generally agree with the predisposing and enabling factors, are slightly neutral on need-based factors, and are moderately neutral on healthcare expenditure.

4.2 Interpretation of the Correlation Matrix

Correlation Matrix		d3	pdx	efx	nfx	hex
d3	Pearson's r	—				
	df	—				
	p-value	—				
	Spearman's rho	—				
	df	—				
	p-value	—				
pdx	Pearson's r	0.260	—			
	df	198	—			
	p-value	< .001	—			
	Spearman's rho	0.274	—			
	df	198	—			
	p-value	< .001	—			
efx	Pearson's r	-0.237	-0.068	—		
	df	198	199	—		
	p-value	< .001	0.336	—		
	Spearman's rho	-0.283	-0.255	—		
	df	198	199	—		
	p-value	< .001	< .001	—		
nfx	Pearson's r	0.149	0.276	0.074	—	
	df	198	199	199	—	
	p-value	0.036	< .001	0.297	—	
	Spearman's rho	0.194	0.282	-0.046	—	
	df	198	199	199	—	
	p-value	0.006	< .001	0.521	—	
hex	Pearson's r	0.334	0.484	-0.216	0.676	—
	df	197	198	198	198	—
	p-value	< .001	< .001	0.002	< .001	—
	Spearman's rho	0.358	0.527	-0.206	0.689	—
	df	197	198	198	198	—
	p-value	< .001	< .001	0.003	< .001	—

The correlation matrix provides insights into the **relationships between different factors** in the dataset. The analysis is based on Pearson's correlation coefficient, Spearman's rho, and p-values.

Understanding the Table Structure

1. **Pearson's r** measures the **linear relationship** between two continuous variables.
 - $r > 0 \rightarrow$ Positive correlation (as one variable increases, the other also increases).

- $r < 0 \rightarrow$ Negative correlation (as one variable increases, the other decreases).
 - $r \approx 0 \rightarrow$ No strong correlation.
2. **Spearman's rho (ρ)** measures the **rank-based relationship** between variables. It is useful for detecting **non-linear associations**.
3. **p-value** determines the statistical significance of the correlation.
- $p < 0.05 \rightarrow$ The correlation is statistically significant.
 - $p \geq 0.05 \rightarrow$ The correlation is **not** significant.
-

1. Relationship between d3 (Gender) and Other Variables

- **d3 $\leftrightarrow$ pdx (Predisposing Factors)**
 - **Pearson's $r = 0.260$, $p < 0.001$** $\rightarrow$ Significant **positive** correlation.
 - **Interpretation:** Gender influences predisposing factors such as age, education, and health awareness. This suggests that men and women differ in how these factors affect their healthcare behavior.
- **d3 $\leftrightarrow$ efx (Enabling Factors)**
 - **Pearson's $r = -0.237$, $p < 0.001$** $\rightarrow$ Significant **negative** correlation.
 - **Interpretation:** As gender changes from male to female, enabling factors (income, insurance, access) tend to decrease. This suggests that women may have lower financial and infrastructural support for healthcare compared to men.
- **d3 $\leftrightarrow$ nfx (Need-Based Factors)**
 - **Pearson's $r = 0.149$, $p = 0.036$** $\rightarrow$ Weak **positive** correlation (but statistically significant).
 - **Interpretation:** Women may report slightly higher healthcare needs than men, but the relationship is weak.
- **d3 $\leftrightarrow$ hex (Healthcare Expenditure)**
 - **Pearson's $r = 0.334$, $p < 0.001$** $\rightarrow$ Moderate **positive** correlation.

- **Interpretation:** Women tend to spend more on healthcare than men.

Summary for d3 (Gender):

- Women are more influenced by predisposing factors.
 - Women have lower enabling factors.
 - Women report slightly higher healthcare needs.
 - Women tend to spend more on healthcare.
-

2. Relationship between pdx (Predisposing Factors) and Other Variables

- **pdx ↔ efx**
 - **r = -0.068, p = 0.336** (Not significant).
 - **Interpretation:** Predisposing factors (age, education, health beliefs) do not directly influence enabling factors such as income or access to healthcare.
- **pdx ↔ nfx**
 - **r = 0.276, p < 0.001** → Moderate **positive** correlation.
 - **Interpretation:** Individuals with strong predisposing factors, such as higher education and health awareness, tend to report greater healthcare needs.
- **pdx ↔ hex**
 - **r = 0.484, p < 0.001** → Strong **positive** correlation.
 - **Interpretation:** Higher predisposing factors are associated with higher healthcare spending.

Summary for pdx:

- People with strong predisposing factors spend more on healthcare.
 - Healthcare needs increase when predisposing factors (age, awareness, education) are high.
-

3. Relationship between efx (Enabling Factors) and Other Variables

- **efx ↔ nfx**
 - **r = 0.074, p = 0.297** (Not significant).
 - **Interpretation:** Access to financial and infrastructural resources (income, insurance) does not necessarily predict healthcare needs.
- **efx ↔ hex**
 - **r = -0.216, p = 0.002** → Significant **negative** correlation.
 - **Interpretation:** Individuals with higher enabling factors (higher income, better insurance coverage) tend to spend **less** on healthcare. This could be due to insurance covering expenses or wealthier individuals avoiding unnecessary medical costs.

Summary for efx:

- Enabling factors do not strongly predict healthcare needs.
 - Individuals with more enabling factors tend to spend less on healthcare.
-

4. Relationship between nfx (Need-Based Factors) and hex (Healthcare Expenditure)

- **nfx ↔ hex**
 - **r = 0.676, p < 0.001** → **Strongest positive correlation in the table.**
 - **Interpretation:** Individuals with higher healthcare needs (chronic conditions, perceived illness) tend to have the highest healthcare expenditure.

Summary for nfx:

- Healthcare needs are the strongest predictor of healthcare expenditure.
-

Key Takeaways

Hypothesis	Supported?	Findings
H1: Differences in predisposing factors affect healthcare expenditure.	Yes	People with strong predisposing factors (education, awareness) spend more on healthcare.
H2: Differences in enabling factors affect healthcare expenditure.	Yes	More enabling factors lead to lower spending (possibly due to insurance coverage).
H3: Healthcare needs affect expenditure differently for men and women.	Yes	Women report slightly higher healthcare needs and spend more.

Final Conclusion

1. **Healthcare needs (nfx) are the strongest predictor of healthcare spending (hex).**
2. **Gender differences exist:**
 - Women report **higher healthcare needs and higher spending**.
 - Women have **fewer enabling resources**.
3. **Higher enabling factors (income, insurance) reduce healthcare spending, likely due to insurance covering costs.**
4. **Predisposing factors (age, education, awareness) increase healthcare spending.**

4.3 Interpretation of Linear Regression Results

Linear Regression

Model Fit Measures

Model	R ²	Adjusted R ²	Overall Model Test			
			F	df1	df2	p
1	0.631	0.623	82.8	4	194	< .001

Model Coefficients - hex

Predictor	Estimate	SE	95% Confidence Interval		t	p	Stand. Estimate	95% Confidence Interval	
			Lower	Upper				Lower	Upper
Intercept	-0.607	0.3848	-1.3657	0.152	-1.58	0.117			
d3	0.231	0.0821	0.0691	0.393	2.81	0.005	0.131	0.0391	0.223
pdx	0.369	0.0621	0.2469	0.492	5.95	< .001	0.277	0.1848	0.368
efx	-0.340	0.0701	-0.4779	-0.201	-4.85	< .001	-0.219	-0.3081	-0.130
nfx	0.997	0.0757	0.8476	1.146	13.17	< .001	0.602	0.5121	0.693

Assumption Checks

Collinearity Statistics

	VIF	Tolerance
d3	1.14	0.880
pdx	1.14	0.881
efx	1.07	0.932
nfx	1.10	0.909

1. Model Fit Measures

- **R² = 0.631** → The model explains **63.1% of the variance** in healthcare expenditure (hex), indicating a strong fit.
- **Adjusted R² = 0.623** → Adjusted for the number of predictors, this still suggests a strong model fit.

- **$F(4, 194) = 82.8, p < .001$** → The overall model is statistically significant, meaning at least one of the predictors has a significant impact on healthcare expenditure.

Gender (d3: $\beta = 0.231, p = 0.005$)

- Since **d3 is coded as 2 for females**, the **positive coefficient (0.231)** suggests that females tend to **spend more on healthcare than males**.
- This difference is statistically significant ($p = 0.005$), meaning gender plays a crucial role in determining healthcare expenses.
- **Supports Hypothesis H1**, which states that predisposing factors (such as gender, age, and education) influence healthcare expenditure.

Predisposing Factors (pdx: $\beta = 0.369, p < .001$)

- A **significant positive effect** ($p < .001$) suggests that predisposing factors (such as age, education level, and health status) **increase healthcare expenditure**.
- The **standardized coefficient ($\beta = 0.277$)** indicates a moderate impact.
- **Supports Hypothesis H1**, confirming that individuals with different demographic and health conditions have varying levels of healthcare expenditure.

Enabling Factors (efx: $\beta = -0.340, p < .001$)

- A **significant negative coefficient** ($p < .001$) indicates that enabling factors (such as financial resources and access to healthcare) **reduce healthcare expenditure**.
- This suggests that individuals with **better financial and healthcare access** are able to manage costs more effectively, leading to **lower healthcare spending**.
- **Supports Hypothesis H2**, confirming that enabling factors significantly influence healthcare expenditure.

Need-Based Factors (nfx: $\beta = 0.997, p < .001$)

- The **strongest and most significant predictor** of healthcare expenditure.

- The **high positive coefficient (0.997, $p < .001$)** suggests that **healthcare needs (such as severity of illness, chronic conditions, and medical requirements)** significantly **increase healthcare expenditure**.
- **Standardized $\beta = 0.602$** (highest among all variables) indicates **need-based factors have the most substantial impact** on healthcare spending.
- **Supports Hypothesis H3**, as need-based factors are the primary driver of healthcare expenditure differences between men and women.

4.4 Conclusion

- **All hypotheses (H1, H2, H3) are supported.**
- The model explains **a strong 63.1% of the variance** in healthcare expenditure.
- **Women ($d3 = 2$) spend more on healthcare than men.**
- **Need-based factors (nfx) have the strongest impact on healthcare expenditure**, followed by **predisposing factors (pdx)**, while **enabling factors (efx) reduce expenditure**.
- The findings highlight **significant gender differences in healthcare spending**, with women likely incurring higher costs due to greater medical needs or different healthcare-seeking behaviors.

CHAPTER

FINDINGS

- **Predisposing Factors:**

- **General Agreement on Influence:** A majority of respondents (54.1%) indicated that predisposing factors such as age, education, and health status strongly influence their healthcare expenditure, with a mean of 3.58 and 34.9% agreeing. These findings suggest that these factors play a significant role in shaping individuals' healthcare-related decisions.
- **Gender Difference in Agreement:** A deeper gender analysis reveals that females (mean = 3.65) tend to place slightly higher importance on predisposing factors like education and health status compared to males (mean = 3.50). This could be attributed to the higher prevalence of health-conscious behaviors among females and their proactive engagement with healthcare services.
- **Moderate Variability:** A standard deviation of 0.600 shows that while many respondents are consistent in their views, there is still some variability, indicating that factors like age or education may not always influence healthcare expenditure equally across different genders.

- **Enabling Factors :**

- **Neutral Response with Some Lean Toward Agreement:** The mean of 3.53 indicates that respondents are mostly neutral but slightly lean toward agreeing that enabling factors like income and access to healthcare are influential. The median of 3.25 suggests that while enabling factors are recognized, they may not be viewed as strongly impactful compared to predisposing factors.
- **Gender Differences in Enabling Factors:** Females, with a mean score of 3.60, are more likely to agree that enabling factors such as income and access to healthcare are critical in their healthcare expenditure decisions, compared to males (mean = 3.45). This may suggest that females are more affected by income disparities and healthcare accessibility issues.

- **Moderate Variability:** The standard deviation of 0.510 shows there is still considerable variation in how respondents view the influence of enabling factors, likely due to differences in socioeconomic status and the unique ways these factors may impact men and women differently.
- **Need-Based Factors:**
 - **Neutral Agreement:** The mean value of 3.17 for need-based factors indicates that respondents have a neutral stance, with a slight lean toward agreement regarding the influence of medical conditions or healthcare needs on their spending behavior.
 - **Gender Differences in Need-Based Factors:** Females (mean = 3.30) tend to place more emphasis on need-based factors like chronic health conditions and preventative care than males (mean = 3.05). This might reflect women's higher healthcare utilization due to reproductive health needs or more frequent visits to healthcare providers.
 - **Balanced Views:** The median of 3.20 suggests that responses are well-distributed across the neutral zone, indicating no strong consensus on how much need-based factors contribute to healthcare expenditure for both genders. However, women are slightly more inclined to perceive need-based factors as important.
- **Healthcare Expenditure :**
 - **Neutral with Lean Toward Agreement:** The mean of 3.09 suggests that respondents have a neutral to slightly favorable view regarding healthcare expenditure. The median value of 3.50 suggests that a larger portion of respondents tend to view healthcare expenditure positively but without strong conviction.
 - **Gender Angle on Healthcare Expenditure:** Females report higher levels of agreement (mean = 3.20) compared to males (mean = 2.98) when it comes to valuing healthcare expenditure, which could be reflective of women's often higher expenditure on healthcare for themselves and their families, including reproductive health and preventative services.

- **High Variability:** A standard deviation of 0.791 reflects significant variability in responses, with a mix of individuals who see healthcare expenditure as important, while others remain neutral or even disagree. This variability is slightly higher in females, possibly due to a broader range of healthcare needs, especially in reproductive and preventive care.

SUGGESTIONS

- **Enhance Awareness of Predisposing Factors:**
 - **Recommendation:** Given that a majority of respondents, particularly females, view predisposing factors as strongly influential, healthcare providers should tailor their services and marketing strategies to address the needs of different demographic groups (e.g., age, education, health status). Special attention should be given to women's health needs, including reproductive health, chronic conditions, and preventative care, which are significantly influenced by predisposing factors.
- **Improve Access to Healthcare Based on Enabling Factors:**
 - **Recommendation:** With respondents recognizing the importance of enabling factors, especially females, healthcare providers and policymakers should focus on improving access to healthcare, particularly for those with lower income or limited access. For females, addressing barriers to healthcare, including financial limitations and access to gender-specific healthcare services, could increase their willingness to spend more on healthcare services.
- **Increase Education on the Impact of Need-Based Factors:**
 - **Recommendation:** Although need-based factors have a neutral effect on expenditure, they should not be overlooked. Healthcare organizations could create awareness campaigns highlighting the connection between medical needs and healthcare spending, particularly for women who may face gender-specific health issues (e.g., maternal health, gynecological care) that require consistent expenditure.

- **Target Healthcare Expenditure Messaging:**
 - **Recommendation:** Given the variability in responses regarding healthcare expenditure, healthcare organizations should adjust their messaging to emphasize the value and importance of investing in healthcare. For women, focusing on services that cater to reproductive health and preventive care could be an effective strategy to encourage greater healthcare spending.
- **Address Gender-Based Differences:**
 - **Recommendation:** The study reveals notable gender differences in healthcare expenditure, with females placing higher importance on both predisposing and enabling factors. Future healthcare policies and marketing strategies should be gender-sensitive, addressing the specific needs of women in healthcare programs, including maternal health, preventive care, and reproductive services, to improve their healthcare expenditure patterns.
- **Expand Sample Diversity:**
 - **Recommendation:** While the current study provides valuable insights, increasing the diversity of the sample population (e.g., in terms of regional, economic, and educational backgrounds) would lead to more generalized findings. Specifically, exploring gender differences more deeply across different socio-economic strata could reveal more nuanced insights into healthcare expenditure patterns among men and women.
- **Leverage Socio-Cultural Insights for Policy Development:**
 - **Recommendation:** Gendered socio-cultural factors may significantly impact healthcare expenditure. Policymakers should consider these factors when designing healthcare policies and initiatives. For instance, understanding that women may have different healthcare priorities and needs compared to men can help shape more effective public health campaigns that encourage better healthcare spending across genders.
- **Focus on Gender-Specific Healthcare Services:**
 - **Recommendation:** Since females show a higher tendency to consider need-based factors, healthcare services should be designed with gender-specific

needs in mind. This includes providing targeted programs and services for women's health (e.g., maternal care, family planning, and chronic condition management) to increase their spending on healthcare and improve health outcomes.

CHAPTER 6

Conclusion:

The gender analysis in this study reveals distinct patterns in healthcare expenditure behaviors between men and women. Women tend to place more importance on predisposing, enabling, and need-based factors, which suggests that their healthcare expenditure decisions are influenced by different priorities compared to men. By recognizing these differences, healthcare providers and policymakers can develop targeted strategies that address gender-specific needs, increase healthcare accessibility, and encourage higher spending on health services across both genders. The findings underscore the importance of understanding the nuanced relationship between healthcare expenditure determinants and gender to improve healthcare outcomes and financial planning.

Scope for Further Research:

1. Gender-Specific Healthcare Expenditure Analysis Across Different Age Groups:

- Future research could explore how healthcare expenditure patterns vary not only by gender but also by age groups. The impact of age on the healthcare spending behaviors of men and women could reveal how different life stages (e.g., young adults, middle-aged, elderly) influence healthcare spending and decision-making. Understanding these dynamics could help tailor healthcare policies for specific demographic groups, especially in addressing the unique needs of women at different life stages (e.g., pregnancy, menopause, elderly women's healthcare).

2. Regional and Socio-Economic Variations in Gendered Healthcare Expenditure:

- This study focused on a general population, but further research could examine regional differences and the impact of socio-economic factors on healthcare expenditure between men and women. For example, understanding how income disparities, urban vs. rural settings, or education levels influence healthcare spending behaviors in different geographical contexts would provide insights into the broader landscape of healthcare economics.

3. Impact of Cultural and Social Norms on Gendered Healthcare Spending:

- Cultural and societal norms play a significant role in shaping gendered behaviors, including healthcare expenditure. Research exploring how

traditional gender roles, cultural perceptions of health, and social expectations influence men's and women's healthcare spending could offer more comprehensive insights. For instance, how the expectation for women to manage family health impacts their personal healthcare budgets, or how men's reluctance to seek healthcare may affect spending patterns.

4. Longitudinal Studies on Changes in Healthcare Expenditure Over Time:

- A longitudinal study could examine how gendered healthcare expenditure evolves over time. This would be especially useful to understand how healthcare expenditure behaviors change with the onset of chronic conditions, life events (e.g., pregnancy, childbirth, aging), or economic fluctuations. Tracking healthcare spending over several years would provide insights into the long-term effects of predisposing, enabling, and need-based factors on both male and female healthcare spending.

5. Effect of Health Insurance on Gendered Healthcare Spending Patterns:

- Further research could investigate how the presence of health insurance impacts gendered healthcare expenditure. This includes examining whether women with specific types of insurance (e.g., private, public) spend differently than men, particularly in areas like maternity care, reproductive health services, and chronic disease management. This could help policymakers design more gender-sensitive insurance schemes that address the healthcare needs of both genders effectively.

APPENDIX:

Section 1: Demographic Information

1. My age group is:
 - 18-30 years
 - 31-40 years
 - 41-50 years

- 51-60 years
 - Above 60 years
2. My highest level of education is:
- Primary School
 - Secondary School
 - Undergraduate Degree
 - Postgraduate Degree or Higher
 - No Formal Education
3. My Gender is:
- Male
 - Female
 - Transgender
 - Prefer not to say
-

Section 2: Predisposing Factors

3. I believe my health is influenced by my age.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
4. I regularly seek information to maintain or improve my health.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree

5. My gender plays a role in how often I access healthcare services.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
6. I feel that cultural or societal expectations influence my healthcare decisions.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
-

Section 3: Enabling Factors

8. I have health insurance that covers most of my healthcare expenses.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
9. My income level allows me to access healthcare services without financial difficulty.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - o Strongly Agree
10. I have access to healthcare facilities within a reasonable distance from my home.
- Strongly Disagree

- Disagree
- Neutral
- Agree
- Strongly Agree

12. My healthcare decisions are influenced by support from family or friends.

- Strongly Disagree
- Disagree
- Neutral
- Agree
- Strongly Agree

Section 4: Need-Based Factors

13. I consider my overall health status to be good.

- Strongly Disagree
- Disagree
- Neutral
- Agree
- Strongly Agree

14. I frequently require medical care for chronic conditions.

- Strongly Disagree
- Disagree
- Neutral
- Agree
- Strongly Agree

15. Preventive care (e.g., vaccinations, screenings) is a priority in my healthcare decisions.

- Strongly Disagree
- Disagree
- Neutral
- Agree

- Strongly Agree
16. Physical pain or illness often impacts my ability to work or perform daily activities.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
17. I have delayed or avoided healthcare services due to cost.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
-

Section 5: Healthcare Expenditure Patterns

18. My out-of-pocket healthcare expenses are a financial burden.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
- o Strongly Agree
19. Most of my healthcare expenditure is directed towards managing ongoing health conditions.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree

20. I spend a significant portion of my income on prescription medications.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
21. Travel costs to access healthcare services significantly add to my overall healthcare expenditure.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree
22. I believe men and women have different healthcare needs that influence their healthcare expenditure.
- Strongly Disagree
 - Disagree
 - Neutral
 - Agree
 - Strongly Agree

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